CCLA CALIFORNIA CLINICAL LABORATORY ASSOCIATION

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February 19, 2026

The Honorable Chris Klomp
Chief Counselor
Office of the Secretary
U.S. Department of Health and Human Services
200 Independence Ave., SW
Washington, DC 20201

The Honorable Thomas Keane, MD, MBA
Assistant Secretary for Technology Policy and
Office of the National Coordinator for Health Information Technology
U.S. Department of Health and Human Services
330 C St SW, Floor 7
Washington, DC 20201

Re: Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care (HHS Health Sector AI RFI) [Docket No. HHS-ONC-2026-0001]

Submitted via Electronic Submission to www.regulations.gov

Dear Deputy Secretary O'Neill and Assistant Secretary Keane,

On behalf of the California Clinical Laboratory Association (CCLA), we appreciate the opportunity to provide comments on the recently issued Request for Information (RFI) entitled *Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care* by the Department of Health and Human Services (HHS) Office of the Deputy Secretary in collaboration with the Assistant Secretary for Technology Policy (ASTP)/Office of the National Coordinator for Health Information Technology.

Our association represents a broad range of clinical laboratories serving patients across California, including large national diagnostic laboratories, specialized and biotechnology-focused laboratories, academic and health system pathology departments, and smaller independent clinical laboratories and service providers. California laboratories operate at the forefront of biotechnology and digital health innovation, and regulatory clarity and reimbursement stability are critical to sustaining responsible AI advancement in clinical diagnostics.

CCLA Responses to RFI Questions

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Regulation
HHS seeks feedback on how current HHS regulations impact AI adoption and use for clinical care.

CCLA Response:

Federal regulatory clarity is essential to enable laboratories to adopt AI safely and at scale. Current federal frameworks—particularly CLIA and FDA oversight—serve distinct and complementary roles. CLIA governs clinical laboratory services, including laboratory developed tests (LDTs) and algorithmic analyses performed as professional services, while FDA oversight applies to regulated medical devices and software marketed by manufacturers. Clear jurisdictional boundaries are necessary to prevent duplicative or misaligned oversight and to ensure continued innovation in laboratory-developed services.

For AI-enabled medical devices, laboratories require clear and actionable manufacturer transparency to support local verification and validation prior to clinical implementation. Transparency should include model development datasets and intended-use populations; performance metrics and validation methodology; ongoing performance monitoring and change-control plans; known limitations and deployment constraints; and configuration and customization parameters. Such disclosures support laboratory oversight, mitigate risks associated with dataset imbalance or model drift, and enhance physician confidence in AI-enabled tools.

CCLA encourages HHS to reevaluate 45 CFR 92.210 (Nondiscrimination in the use of patient care decision support tools) to ensure that liability for algorithmic bias is appropriately allocated between AI developers and clinical end users. The current framework risks placing disproportionate responsibility on clinicians and laboratories for potential bias originating in vendor training data or model design. Regulatory clarity in this area will promote responsible adoption without discouraging innovation.

The Clinical Laboratory Improvement Amendments (CLIA) remain the primary regulatory framework for laboratory services, including laboratory services that examine and analyze digital laboratory information. However, CLIA regulations were promulgated decades ago, before the proliferation of software, robust networking tools, and AI-enabled diagnostics that now enhance diagnostic workflows. Targeted regulatory updates to CLIA would advance innovation and clarify oversight responsibilities. Specifically:

(1) The definition of "laboratory" under CLIA should be amended to expressly state that laboratories include facilities that examine digital laboratory information, such as previously sequenced DNA or digital images of slides, and that "algorithmic" examinations are included among the functions of a laboratory. Clarifying that CLIA is the applicable regulatory framework for algorithmic-only laboratory services will remove regulatory ambiguity and support continued innovation.

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(2) CMS should codify into regulation its enforcement discretion policy that permits pathologists and other laboratory personnel to review digital laboratory data, digital results, and digital images remotely under the primary site's CLIA certificate.

(3) HHS should reinstate or replace the former Clinical Laboratory Improvement Advisory Committee (CLIAC), or establish a comparable advisory body, to provide expert guidance as AI transforms laboratory medicine.

For specific wording guidance related to CLIA regulatory updates, we defer to the language recommendations outlined in the ACLA response to this Health Sector AI RFI.

Reimbursement
HHS seeks feedback on payment policy changes that ensure payers have the incentive and ability to promote access to high-value AI clinical interventions, foster competition among clinical care AI tool builders, and accelerate access to and affordability of AI tools for clinical care.

CCLA Response:

Sustainable adoption of AI-enabled laboratory services requires predictable, value-based reimbursement pathways that reflect the clinical benefit and the substantial investment required to develop, validate, and maintain algorithmic analyses—even when no new specimen is collected. AI-enabled algorithmic services improve diagnostic precision, better inform treatment decisions, and reduce unnecessary downstream testing or ineffective therapies.

Absent viable coding and payment pathways, innovative AI-enabled diagnostics may migrate away from Medicare beneficiaries, exacerbating disparities in access and limiting innovation within federally funded programs.

Applicable clinical laboratory services may be reimbursed under the Medicare Clinical Laboratory Fee Schedule (CLFS) or the Medicare Physician Fee Schedule (PFS) when physician professional time is required. The clinical value of AI-enabled services and the resources required to furnish them should be appropriately reflected in reimbursement under both payment systems.

Federal engagement is needed to align coding and payment policy with regulatory frameworks. The Proprietary Laboratory Analysis (PLA) code pathway has historically supported innovative laboratory services, including algorithmic-only analyses. Recent changes to PLA guidance that render algorithmic-only services ineligible for PLA coding create uncertainty and risk impeding innovation. CMS should work with the American Medical Association (AMA) to ensure that appropriate coding pathways remain available for algorithmic-only laboratory services.

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Additionally, AI and automated systems should not be used to deny or impede patient access to services automatically. AI-driven utilization management tools and automated edits must include meaningful human clinical review, transparent denial rationales, and clear appeal pathways to ensure that patients are not denied medically necessary care.

HHS should also ensure that Medicare Advantage plans implement National Coverage Determinations (NCDs) consistently with Medicare fee-for-service standards and do not apply AI-driven utilization tools or local coverage criteria that restrict access beyond established federal coverage policy.

Research & Development
HHS seeks input on ways in which HHS may invest in research & development (including public-private partnerships and cooperative research and development agreements (CRADAs)) to integrate AI in care delivery and create new, long-term market opportunities that improve the health and wellbeing of all Americans.

CCLA Response:

HHS should prioritize funding for public-private partnerships, CRADAs, and grant programs that build diverse, high-quality validation datasets; support interoperable data standards; enable real-world evidence (RWE) generation; develop synthetic data governance frameworks; and expand external proficiency testing models for AI-enabled laboratory services under CLIA.

Investments in standardized benchmarking, external validation programs, and workforce training will reduce implementation risk and accelerate trustworthy AI deployment while maintaining quality and patient safety. Federal funding should also recognize and offset the operational costs laboratories incur when standardizing data and adopting interoperable formats.

AI research and innovation in clinical laboratories advance most effectively when reimbursement is stable and regulatory expectations are clear, consistent, and technology-neutral. Predictable policy environments reduce innovation risk and promote responsible deployment of AI tools that enhance patient care.

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Thank you for the opportunity to provide commentary to HHS. CCLA looks forward to working collaboratively with HHS to promote responsible AI implementation within the clinical laboratory community and to ensure continued patient access to high-quality diagnostics.

Sincerely,

Kristi Foy

Kristi Foy
Executive Director
California Clinical Laboratory Association